



LEVITATTE

LEARNING & DEVELOPMENT

Elevating Healthcare Through People.

A structured 90-day learning and development programme

END-TO-END LEARNING & DEVELOPMENT SOLUTIONS FOR HEALTHCARE ORGANISATIONS

Prepared for the leadership of Dr. Rela Institute & Medical Centre, Chromepet, Chennai

AUGUST 2026

Competent. Compassionate. Confident. Accountable. Patient-First.

Private and confidential. Prepared exclusively for the leadership of Dr. Rela Institute & Medical Centre.

THE ENGAGEMENT

90

DAYS

39

SESSIONS

156

FACILITATOR-LED HOURS

12

CUSTOMISED MODULES

30

PARTICIPANTS PER BATCH

Sessions and hours are upper bounds: 36 to 39 sessions and 144 to 156 hours, subject to start date and hospital holidays.

Rs 2,00,000

per month, fixed. GST as applicable.

Rs 6,00,000 for the 90-day engagement

Every interaction is part of care.

Every patient interaction shapes the reputation of a hospital. From the first telephone call and the registration counter through treatment coordination, billing and discharge, patients and their families expect clarity, compassion, professionalism and confidence.

Across a full episode of care the treating consultant is present for only some of the patient's time in the building. The rest belongs to front office, patient relations, billing, nursing and clinical support, housekeeping and security, who hold the experience together at moments the clinician never sees. Clinical excellence can be concentrated in specialists; the experience of being cared for cannot, because it is assembled touchpoint by touchpoint. That consistency is a capability, and it is trainable.

Quality is not what the hospital intends. It is what the patient remembers about the way they were treated at every touchpoint.

WHO WE ARE

Levitatte brings a founder with more than twenty-five years in behavioural, communication and service capability building, grounded in NLP and organisational behavioural psychology, with prior healthcare engagements at Fortis and Cloud9 Hospitals. Industry-specific knowledge of hospital floors shapes every module: the content is customised against your own policies, SOPs and service standards rather than delivered from a generic library.

WHAT IT SUPPORTS

A programme of this kind supports more confident employees, a more consistent patient experience, better complaint handling and service recovery, improved teamwork, stronger accountability, more effective supervisors, and a culture of continuous learning.

WHAT THE PROGRAMME BUILDS

AWARENESS & SELF-EFFICACY

Self-awareness of the standard one is setting, and the confidence to hold it without being supervised into it.

PERSONALITY & IMAGE

Professional presence, bearing and grooming: the image the hospital projects before a word is spoken.

EMOTIONAL INTELLIGENCE

Empathy enhancement sessions, self-regulation under pressure, and the protection of patient dignity and privacy.

SERVICE EXCELLENCE

Proactive service, quality at every touchpoint, and the honest management of expectations and delays.

SERVICE RECOVERY & CRISIS MANAGEMENT

Complaint handling, conflict resolution, de-escalation, incident reporting and closing the loop with families.

COORDINATION & COMMUNICATION

Interdepartmental connection, effective handovers and escalation protocol that holds across shifts.

RESILIENCE & SELF-MOTIVATION

Stress management, compassion fatigue, accountability and the discipline to sustain a standard on a hard day.

BRAND AWARENESS

Every employee carries the hospital's brand. Each interaction either builds it or quietly spends it.

FORECASTED THINKING

Reading the day ahead: anticipating load, delay and risk in daily operations instead of reacting to them.

The twelve modules, and why in this order

The sequence runs from awareness to self, then to tools, inner capability, application, complexity, sustainability and finally multiplication. Three placements are deliberate.

MODULE	FOCUS	WEEKS	SESSIONS
M1 Foundation & Orientation	Vision, values, patient rights, conduct, safety protocol, emergency situational awareness, escalation and incident reporting	1	3
M2 Professional Presence	Grooming, bearing, punctuality, personality and professional image, workplace etiquette	2	2
M3 Communication Fundamentals	Verbal and non-verbal, active listening, telephone etiquette, clarity under pressure	2–3	4
M4 Emotional Intelligence & Empathy	Self-awareness, self-regulation, empathy enhancement sessions, patient dignity and privacy	4	3
M5 Patient Experience & Standards	First impressions, guiding, proactive service, service excellence, managing expectations and delays	5–6	4
M6 The Patient Journey	Enquiry to discharge, touchpoint by touchpoint, allocated by role	6–7	4
M7 Difficult Situations & Recovery	Difficult news, complaints, service recovery, conflict resolution, de-escalation, crisis management	7–9	5
M8 Personal Effectiveness & Resilience	Stress management, compassion fatigue, self-motivation, self-efficacy, priorities, accountability	9	2
M9 Teamwork & Coordination	Handovers, internal-customer mindset, interdepartmental coordination, escalation discipline	10	3
M10 Leadership & Supervisory	Huddles, coaching, feedback, conflict resolution, underperformance	11–12	4
M11 Train-the-Trainer	Adult learning, facilitation, assessment, reinforcement	12–13	3
M12 Consolidation & Review	Post-assessment, effectiveness dashboard, management review	13	2
Total	13 weeks, 3 sessions per week	1–13	39

ALLOCATION Every group takes the foundation modules; M6 is allocated by role, M10 is for supervisors and department heads, M11 for the nominated internal trainers.

– PROFESSIONAL PRESENCE EARLY

the patient judges before a word is spoken, and it is the quickest change to become visible, which earns the programme credibility for the harder work that follows.

– DIFFICULT SITUATIONS LATE, RESILIENCE STRAIGHT AFTER

de-escalation depends on active listening, emotional regulation and agreed standards already being in place. Teaching complaint handling first is the most common design error.

– SUPERVISORS AFTER THEIR TEAMS, TRAIN-THE-TRAINER LAST

a supervisor cannot reinforce a standard they did not sit through. Trained staff returning to untrained supervisors regress.

COHORTS Weeks 1 to 10 run frontline and staff batches, with supervisors attending alongside their own teams rather than in a separate senior run. Weeks 11 and 12 take the supervisory and leadership cohort, Weeks 12 and 13 the internal-trainer group. Induction is built during Diagnose and Design and delivered to each new-joiner cohort as they arrive, so it is a standing track rather than a calendar week.

How it runs, and what the hospital receives

HOW WE TEACH

All training is built on the narrative of real experience and real incidents: hospital-based case studies, incident reports, patient-interaction simulations, role plays, scenario discussions, microlearning, on-the-job observation and coaching, with assessment before and after. Employees practise how to do it rather than simply being told. A four-hour session allows a full cycle of concept, demonstration, practice and feedback, of which 105 minutes are structured practice and a further 25 minutes participant case work.

THE FIVE STAGES

01 DIAGNOSE

Management discussions, departmental inputs, employee observation, and review of anonymised feedback and complaints.

02 DESIGN

Every module customised against the hospital's own policies, SOPs and service standards. Induction is built here.

03 DELIVER

Facilitated four-hour sessions: concept, demonstration, practice and feedback, three days a week.

04 REINFORCE

Job aids carried to the point of work, supervisor coaching, and short observation visits during live shifts.

05 MEASURE

Assessment before and after, on-the-job observation, and monthly reporting to management.

WHAT THE HOSPITAL RECEIVES

- Training Needs Analysis report by end of Week 2, competency and skill-gap framework in Week 3, and a training calendar the hospital can extend quarterly
- The induction programme and all twelve customised modules, with facilitator guides, participant materials and role-specific job aids
- Assessment and observation instruments in use from Week 1, consolidated in Week 3
- Attendance records, feedback analysis, monthly training reports, and corrective plans where a standard is not holding
- Management review presentations and a training effectiveness dashboard

HOW EFFECTIVENESS IS REPORTED

- 01 PARTICIPATION AND REACTION** Attendance, completion and participant feedback by batch and by module.
- 02 KNOWLEDGE GAIN** Assessment before and after each module, scored and reported by cohort.
- 03 BEHAVIOURAL CHANGE** On-the-job observation against the same checklists supervisors use on the floor.
- 04 ORGANISATIONAL INDICATORS** Complaint and feedback themes read alongside capability gaps.

The fourth level depends on hospital-held complaint and feedback data, so improvement there cannot be attributed to training alone. We report movement rather than guarantee it.

ALLOCATED BY ROLE · WHAT EACH GROUP KEEPS

Front office and reception

Greeting and telephone aid, waiting-time wording, escalation flowchart

Housekeeping and facility

In-room conduct card, request-and-escalate wording, confidentiality card

Patient relations

Service-recovery card, complaint and closure log, rounding question set

Security

De-escalation ladder, gate-side entry wording, incident escalation aid

Billing and administration

Billing-explanation framework, estimate checklist, de-escalation steps

Supervisors and department heads

Huddle template, observation checklists, feedback structure, coaching questions

Nursing and clinical support

Bedside card, dignity and privacy checklist, handover aid

Managers and leadership

Effectiveness dashboard, monthly review pack, gap-to-action framework

Rs 2,00,000 per month, for ninety days

Rs 2,00,000

per month, fixed. GST as applicable.

Rs 6,00,000 for the 90-day engagement

THE ENGAGEMENT AT A GLANCE

ENGAGEMENT DURATION	90 days (approximately 13 weeks)
TRAINING DAYS	Every Wednesday, Thursday and Friday
SESSION DURATION	4 hours per session
BATCH SIZE	Up to 30 participants per batch
SESSIONS IN THE ENGAGEMENT	Up to 39 (36 to 39 subject to start date and hospital holidays)
FACILITATOR-LED HOURS	Up to 156 hours (144 to 156)

WHAT THE FEE COVERS

One fixed monthly fee of Rs 2,00,000, GST as applicable, for the full ninety days. Nothing in the programme is charged per session, per module or per participant.

The fee is one fixed monthly charge, not a per-session or per-participant rate, so the cost is known at the outset and does not rise as more people are nominated. It covers the diagnose stage, design and customisation of all twelve modules and the induction programme, facilitation, guides, materials and job aids, assessments, observation, records, monthly reporting and management reviews.

TERMS

- Invoices are raised monthly in arrears. GST as applicable.
- The calendar is confirmed jointly before commencement, and sessions missed for hospital reasons are rescheduled within the engagement window where possible.
- Hospital data is treated as confidential.

WHAT WE WOULD NEED FROM THE HOSPITAL

- A single point of contact from HR or L&D, and confirmed batch nominations ahead of each module block
- Participants released for the full four hours, and a training room with projection for up to 30
- Access to anonymised patient-feedback and complaint themes for case studies, and permission for on-the-job observation
- Nominated subject-matter experts for clinical, statutory and policy content, plus the hospital's own SOPs and service standards
- The legal entity name and billing details for invoicing

NOTE *Clinical, statutory and policy-specific training content will be developed or delivered in coordination with authorised hospital representatives and subject-matter experts.*

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We would welcome the chance to walk your team through this in person and to adjust the design in light of what you know about your own floors.

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